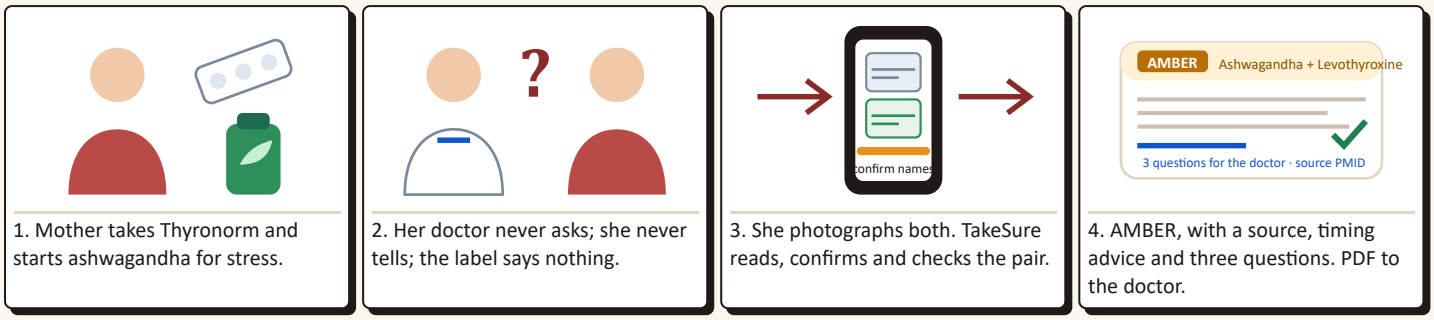
TakeSure *Confidence before taking something*

An Ayurveda × Allopathy herb–drug interaction checker for Indian patients, doctors and the Ministry of Ayush

HACK@BVP 8.0 · Abstract Round
Team Bug Hunters
Dhriti Garg · Shiwani Goyal · Khushi Gupta



1. Mother takes Thyronorm and starts ashwagandha for stress.
2. Her doctor never asks; she never tells; the label says nothing.
3. She photographs both. TakeSure reads, confirms and checks the pair.
4. AMBER, with a source, timing advice and three questions. PDF to the doctor.

Domain: Health & Well-Being (AI4Impact secondary) · SDG 3 · SDG 9 · SDG 10

1 The problem

Two medical systems, no safety check between them. A thyroid patient on Thyronorm adds ashwagandha; a transplant patient on tacrolimus drinks giloy kadha; a cardiac patient on warfarin takes guggul. Each pair can cause harm, yet the doctor rarely asks, the patient rarely tells, the chemist has no tool and the label says nothing. Medscape, Drugs.com and 1mg check only allopathic pairs; Western herb databases are English-only, paid and do not know Indian brands.

37% of hospital patients use allopathic and herbal medicines together

65% of herbal users do not know the dose, side effects or interactions

19% of Indian patients ever disclose herbal use to their doctor

1,473 ASU&H adverse-event reports received nationally since 2018

2 Proposed solution

TakeSure is a phone-first safety layer between the two systems. The patient photographs the allopathic prescription and the Ayurvedic product label (or types the names). TakeSure reads them, confirms every name with the user, maps Indian brands to generics and products to their herbs, checks every herb–drug pair against a curated, cited knowledge base, and returns a plain-language verdict in Hindi or English with sources, timing advice and three questions to ask the doctor. One tap produces a one-page PDF for the doctor and a pre-filled suspected-ADR report in the Ministry of Ayush format for the Ayush Suraksha portal.

> *The verdict comes from published data; AI only explains it.*

3 Approach

1. **Scan.** OCR (Tesseract eng+hin) reads both photos; every name is shown back for confirmation; images deleted after reading
2. **Match.** Bridge dictionary + fuzzy matching: Thyronorm → levothyroxine; Chyawanprash → amla, ashwagandha…; Sanskrit/Hindi/Latin synonyms
3. **Decide.** Deterministic rule engine over a versioned knowledge base; every row carries a source (DOI/PMID), severity and evidence grade A/B/C
4. **Explain.** LLM rewrites only the matched rows into simple Hindi/English with schema-locked output; it cannot add, remove or re-grade an interaction
5. **Report.** Doctor PDF with citations; one-tap ADR form for the Ayush pharmacovigilance network; co-use analytics dashboard

Verdicts GREEN · AMBER · RED · NOT IN DATABASE — Worst row wins; an unknown pair is never shown as green; grade-C evidence caps at AMBER; the answer is identical with the AI switched off.

Stack. Next.js + TypeScript PWA (Hindi/English UI) · FastAPI + Pydantic + SQLAlchemy REST API · PostgreSQL in production, SQLite offline · Tesseract OCR · RapidFuzz matching (≥85 accept, 70–85 ask the user) · Groq LLM with JSON-schema output at temperature 0 · ReportLab PDFs. No model training: a data + rules product that runs fully offline on a laptop. **Knowledge base.** 40–60 high-frequency herb × drug pairs (15 common herbs × 12 drug classes) curated from PubMed, NIH LiverTox, MSKCC About Herbs, WHO monographs and the Ayurvedic Pharmacopoeia of India, graded on the Stockley's model. Decision support only: “ask your doctor”, never diagnosis or dosing; disclaimer on every screen and PDF.

4 Expected impact

Patients & caregivers. A sourced answer before the first dose, in their own language, instead of silence or a guess. Free for citizens.

Doctors, vaidyas & chemists. Finally learn what patients actually take; a QR on the chemist's bill or an API inside hospital and telemedicine systems makes the check routine.

Ministry of Ayush. Structured, reproducible ADR reports and real co-use data from the catchment of its 99 peripheral pharmacovigilance centres.

Novelty. Nothing public today checks an Ayurvedic product against an allopathic prescription. TakeSure builds the missing bridge table: Indian brand names × Ayurvedic herb synonyms × graded evidence, open and auditable. Honest by design: grades, timing gaps instead of alarms, “not in database” instead of guesses.